

**Clinical Log, Daily Notes, and Rotation Deliverables:
Competency-Based Examples and Essentials Mapping**

Date: 02/20/26 (Clinical day 14)

Today's goal: To independently assess patients and develop a comprehensive management plan with minimal supervision.

Start and End time: 7 AM – 5 PM

Total hours: 10 hrs

Daily Chronicle

AM Structure:

- Received handoff and reviewed overnight events, labs, and imaging.
- Conducted patient assessments and performed focused physical exams.
- Reviewed ECG and CXR findings and discussed interpretation with preceptor.
- Participated in interdisciplinary rounds and contributed to management planning.
- Updated patient and family regarding clinical status and plan of care.

PM Structure:

- Followed up on lab results and response to treatment.
- Assisted in medication adjustments (HF regimen, insulin management).
- Completed documentation and progress notes.
- Reflected on clinical reasoning and areas for improvement.

Detailed Experiences:

- **Handovers and Consults:**
- **Interdisciplinary Team Collaboration:** NA
- **Patient Communication Encounters:** Explained current diagnosis and treatment plan to the patient who is scheduled for combined Heart cath on Monday.
- **Patient/Family Education:** NA
- **Patient Advocacy Encounters:** NA
- **Evidence-Based Practice Decisions:** Applied GDMT guidelines for heart failure management.
- **Pharmacologic Assessment and Management:** Assisted in adjusting diuretic dose based on volume status and Creat levels.
- **Medication Cost:** NA
- **Care Transitions:** Participated in discharge planning and medication reconciliation.
- **Ethical Challenges:** N/A

- **Autonomous Practice and Leadership Moments: N/A**
- **Social Determinants of Health, DEI, Population Health Assessment and Management: N/A**
- **Skills Development within the context of patient care:** Reviewed and interpreted three ECGs with my preceptor, strengthening my systematic approach (rate, rhythm, axis, intervals, ischemia) and improving confidence in identifying abnormalities.
- **Accounting of daily notes and rotation/semester deliverables**
- **Patient Notes (n = 2/day)**
- **Diagnostic Reasoning (n = 1)**
- **Comprehensive H & P (n = 1)**
- Pt initials from included note, and the situation that prompted the need
- **Reflection on NP practice (n = 1):**

Progress Note 1

HPI:

L.J is a 70 y.o. male with past medical history significant for HFmrEF (40-45%), CAD with prior MI, HTN, HLD, DMII, CKD Stage IV, centrilobular emphysema, recent find of thoracic lymphadenopathy, multiple rib fractures, osteoporosis, hx of falls, and recent admission to Nazareth Hospital 1/22/26-1/27/26 with rib fractures. He presented to the hospital with shortness of breath for a couple of days and was admitted for acute decompensated heart failure.

He received IV diuresis with Bumex IV push with no significant response. On 02/18, he was started on a bumex infusion. Urology was consulted for urinary retention, Urology recommended cudae catheter that was put in successfully by nursing; plan to take out foley once closer to euvolemia. Endocrinology was consulted for assistance with insulin management of diabetes mellitus.

History Interval

Patient wearing Nasal cannula, he reports discomfort in his throat, he describes it as if there is something stuck there. He offers no complaints of chest pain, palpitations, shortness of breath at rest, dizziness, and lightheadedness. He denies Nausea, vomiting or constipation. Foley was removed before examination by nurse.

- 24-hour weight change: down 4 lbs; since admission: down 17 lbs
- Weight today: 135 lbs
- Intake/output: 24-hour -2.4 L; since admission: - 7.7 L
- Telemetry: Sinus rhythm, 70 to 80 beats per minute
- Creatinine today: 2.68 (yesterday: 2.51 and 2.40)

Past Surgical History:

- PR XCAPSL CTRC RMVL INSJ IO LENS PROSTH CPLX WO ECP left 2018
- PR XCAPSL CTRC RMVL INSJ IO LENS PROSTH CPLX WO ECP Right 2019
- PR NEUROPLASTY &/TRANSPOS MEDIAN NRV CARPAL TUNNE B/L 2013

Allergies:

- Lisinopril - Hyperkalemia

ROS:

- **General:** Reports fatigue denies chills/fever or weight changes.
- **HENT:** Reports throat discomfort, no congestion, he is using hearing aids B/L
- **Respiratory:** Reports cough and sputum production, SOB at rest, No wheezing

- **Cardiovascular:** Denies chest pain, palpitations, LE edema B//L
- **Gastrointestinal:** Denies Nausea, vomiting or blood in stool.
- **Genitourinary:** Reports urinary retention yesterday.
- **Musculoskeletal:** Report LE swelling B/L. Denies joint pain, stiffness.
- **Skin:** No rash or itching
- **Neurological:** Reports hx of syncope and Falls, denies headache, dizziness.
- **Psychiatric/Behavioral:** Reports that he anxious.

Inpatient medication:

- Acetaminophen 650 mg PO Q6H PRN
- Albuterol nebulizer solution 2.5 mg via nebulizer Q12H
- Ascorbic acid 500 mg PO Daily
- Aspirin (chewable) 81 mg PO Daily
- Atorvastatin 80 mg PO Daily at bedtime
- Bisoprolol 2.5 mg PO Daily at bedtime
- Calcium–Vitamin D 500 mg–5 mcg PO BID with meals
- Dextrose 12.5 g IV Q15 min PRN
- Dextrose oral gel 15 g PO Q15 min PRN
- Dextrose 25 g IV Q15 min PRN
- Dextrose oral gel 45 g PO Q15 min PRN
- Escitalopram 30 mg PO Daily
- Ezetimibe 10 mg PO Daily
- Ferrous sulfate 325 mg (65 mg elemental iron) PO Q2 days
- Fluticasone–Salmeterol inhaler 250/50 mcg 1 puff inhalation Q12H
- Glycerin topical oral spray (35%) 2 sprays mouth/throat Q4H PRN
- Heparin 5,000 units Subcutaneous Q8H
- Hydralazine 75 mg PO TID
- Insulin aspart (sliding scale) 0–12 units Subcutaneous with meals & bedtime
- Insulin aspart 7 units Subcutaneous Daily with breakfast
- Insulin aspart 8 units Subcutaneous Daily with lunch
- Insulin aspart 8 units Subcutaneous Daily with dinner
- Insulin aspart 6 units Subcutaneous Daily at bedtime
- Insulin glargine (Lantus) 13 units Subcutaneous Daily at bedtime
- Ipratropium nebulizer solution 2.5 mL via nebulizer Q12H
- Isosorbide dinitrate 5 mg PO BID
- Lansoprazole DR 30 mg PO Daily before breakfast
- Multivitamin–minerals 1 tablet PO Daily
- Nicotine patch 14 mg/24 hr Transdermal Daily
- Oxycodone IR 5 mg PO Q4H PRN
- Polyethylene glycol 17 g PO BID PRN
- Sacubitril–Valsartan (Entresto) 97/103 mg PO Q12H
- Senna 8.6 mg PO Q12H
- Sodium bicarbonate 1,300 mg PO Q12H

- Sodium chloride nasal spray 0.65% 2 sprays each nostril Q2H PRN
- Sodium chloride 0.9% flush 0.5–50 mL IV PRN
- Sodium chloride 0.9% KVO 0–10 mL/hr IV Continuous PRN
- Tamsulosin 0.4 mg PO Daily at bedtime

Home Meds:

- ACCU-CHEK FastClix Lancets – Use to check blood glucose – Other – 4 times daily (Dx: E11.9 / previously 250.02; on 4 insulin injections per day)
- Accu-Chek Guide Test Strips – Use to check blood glucose – Other – 4 times daily
- Accu-Chek Guide Device Kit – Use as directed – Other – For blood glucose monitoring
- Alendronate 70 mg PO Once weekly (take on empty stomach; nothing to eat for 30 minutes)
- Ascorbic Acid (Vitamin C) 500 mg PO Daily (0.5 tablet of 1000 mg)
- Breztri Aerosphere (budesonide/glycopyrrolate/formoterol) 160-9-4.8 mcg – Inhalation – 2 puffs Q12H
- Sacubitril-Valsartan (Entresto) 97-103 mg PO BID
- Icosapent Ethyl (Vascepa) 0.5 g PO BID
- Insulin Pen Needles 31G x 8 mm – Use for insulin injections – 5 times daily
- Insulin Syringe U-100 31G x 5/16" (0.5 mL) – Use for insulin injection – Daily
- Empagliflozin (Jardiance) 25 mg tablet – PO Daily in the morning (verify dose; instruction says 0.5 tab vs 1 tab daily — needs clarification)
- Multivitamin (Centrum Adult) 1 tablet PO Daily
- Insulin NPH (Novolin N) 18 units Subcutaneous Daily at bedtime
- Insulin Aspart (NovoLog FlexPen) 20 units Subcutaneous with meals + sliding scale (up to 110 units total daily)
- Acetaminophen 1,000 mg PO Q8H PRN pain (2 tablets of 500 mg)
- Albuterol inhaler 108 mcg (90 mcg base) – 2 puffs Inhalation Q6H PRN wheezing/SOB
- Amlodipine 10 mg PO Daily (morning)
- Aspirin (chewable) 81 mg PO Daily
- Atorvastatin 80 mg PO Daily at bedtime
- Bisoprolol 2.5 mg PO Daily at bedtime (0.5 tablet of 5 mg)
- Bumetanide 1 mg PO BID (increase to 2 mg BID if weight gain >3 lbs or worsening edema)
- Doxycycline hyclate 100 mg PO Daily (morning)
- Escitalopram (Lexapro) 30 mg PO Daily (1.5 tablets of 20 mg)
- Ezetimibe 10 mg PO Daily (morning)
- Ferrous sulfate 325 mg (65 mg elemental iron) PO Every other day
- Hydralazine 75 mg PO TID (patient taking 3 tablets of 25 mg TID)
- Isosorbide dinitrate 5 mg PO BID
- Nicotine patch 14 mg/24 hr Transdermal Daily

- Omeprazole DR 20 mg PO Daily (30 minutes before meal)
- Oxycodone-Acetaminophen (Percocet) 7.5-325 mg PO Q8H PRN pain
- Sodium bicarbonate 1,300 mg PO BID (2 tablets of 650 mg)
- Tamsulosin 0.4 mg PO Daily at bedtime
- Trazodone 50 mg PO Daily at bedtime

Laboratory Data:

HS Troponin	7/21/2024: Troponin T, High Sensitivity (HS TNT) 34; Troponin T, High Sensitivity (HS TNT) 43; Troponin T, High Sensitivity (HS TNT) 41																		
NT proBNP	2/14/2026: NT proBNP 30,940																		
Lipid Screen	Results for orders placed or performed in visit on 04/02/25 LIPID SCREEN Collection Time: 06/02/25 10:37 AM <table><tr><td>Result</td><td>Value</td><td>Ref Range</td></tr><tr><td>Cholesterol, Total</td><td>105</td><td>100 - 199 mg/dL</td></tr><tr><td>Triglycerides</td><td>116</td><td>0 - 149 mg/dL</td></tr><tr><td>HDL Cholesterol</td><td>34 (L)</td><td>>39 mg/dL</td></tr><tr><td>VLDL Cholesterol Cal</td><td>21</td><td>5 - 40 mg/dL</td></tr><tr><td>LDL Cholesterol Calc</td><td>50</td><td>0 - 99 mg/dL</td></tr></table>	Result	Value	Ref Range	Cholesterol, Total	105	100 - 199 mg/dL	Triglycerides	116	0 - 149 mg/dL	HDL Cholesterol	34 (L)	>39 mg/dL	VLDL Cholesterol Cal	21	5 - 40 mg/dL	LDL Cholesterol Calc	50	0 - 99 mg/dL
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VLDL Cholesterol Cal	21	5 - 40 mg/dL																	
LDL Cholesterol Calc	50	0 - 99 mg/dL																	
Hemoglobin A1C	2/14/2026: Hemoglobin A1c 7.4																		
TSH	2/17/2026: TSH 2.15																		
Iron Panel																			

	Results for orders placed or performed during the hospital encounter of 02/14/26 IRON/ TRANSFERRIN/ TRNSAT Collection Time: 02/14/26 9:08 AM		
	Result	Value	Ref Range
	IRON	34 (L)	45 - 182 ug/dL
	TRANSFERRIN	114 (L)	180 - 329 mg/dL
	TRANSFERRIN SATURATION	21	20 - 50 %
Ferritin	2/14/2026: FERRITIN 344.3		
LDH	2/12/2024: LACTATE DEHYDROGENASE 138		

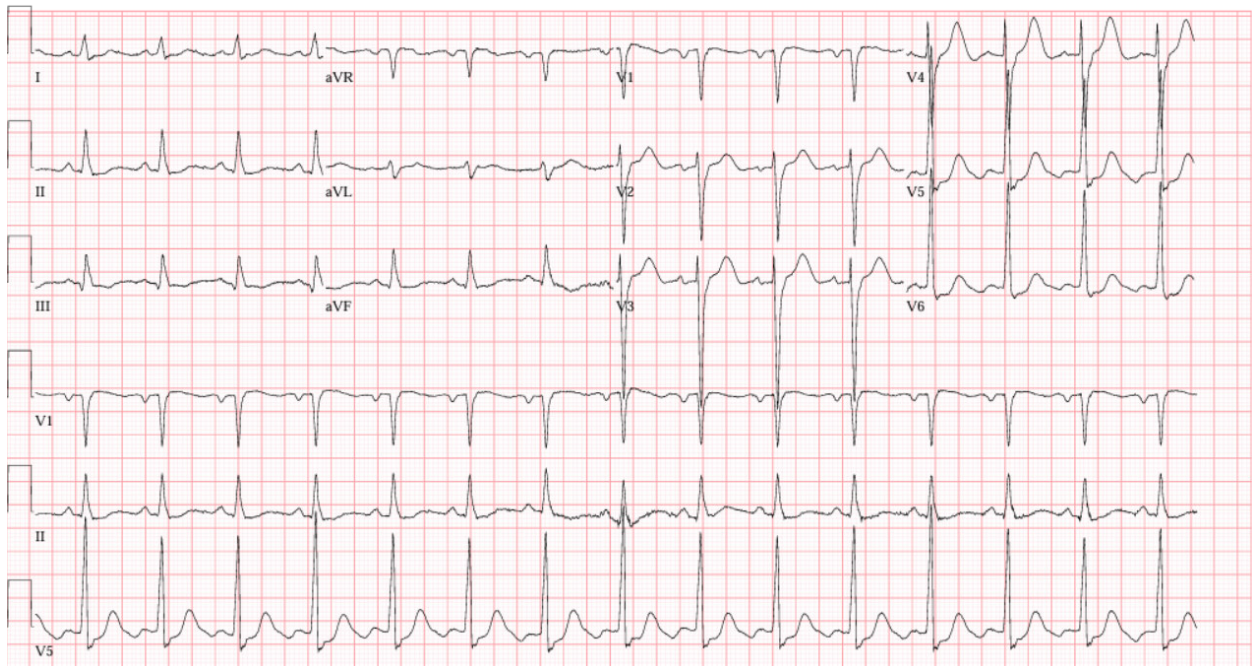
	02/20/26	02/19/26	02/19/26	02/18/26	02/18/26	02/17/26
	0600	1629	0600	1600	0600	1645
SODIUM	136	136	134*	136	133*	133*
POTASSIUM	4.0	3.7	3.6	4.0	4.0	4.0
CHLORIDE	89*	90*	89*	92*	89*	91*
CARBON DIOXIDE	37*	38*	38*	37*	36*	35*
UREA NITROGEN	53*	49*	50*	48*	46*	43*
CREATININE	2.68*	2.51*	2.40*	2.42*	2.30*	2.18*
CALCIUM	8.3*	8.0*	8.1*	8.2*	8.5*	7.8*
GLUCOSE	223*	183*	279*	152*	342*	420*
WBC	--	--	10.7	--	11.2*	--
HEMOGLOBIN	--	--	7.7*	--	7.9*	--
HEMATOCRIT	--	--	22*	--	22*	--

PLATELETS	--	--	230	--	248	--
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Related Imaging:

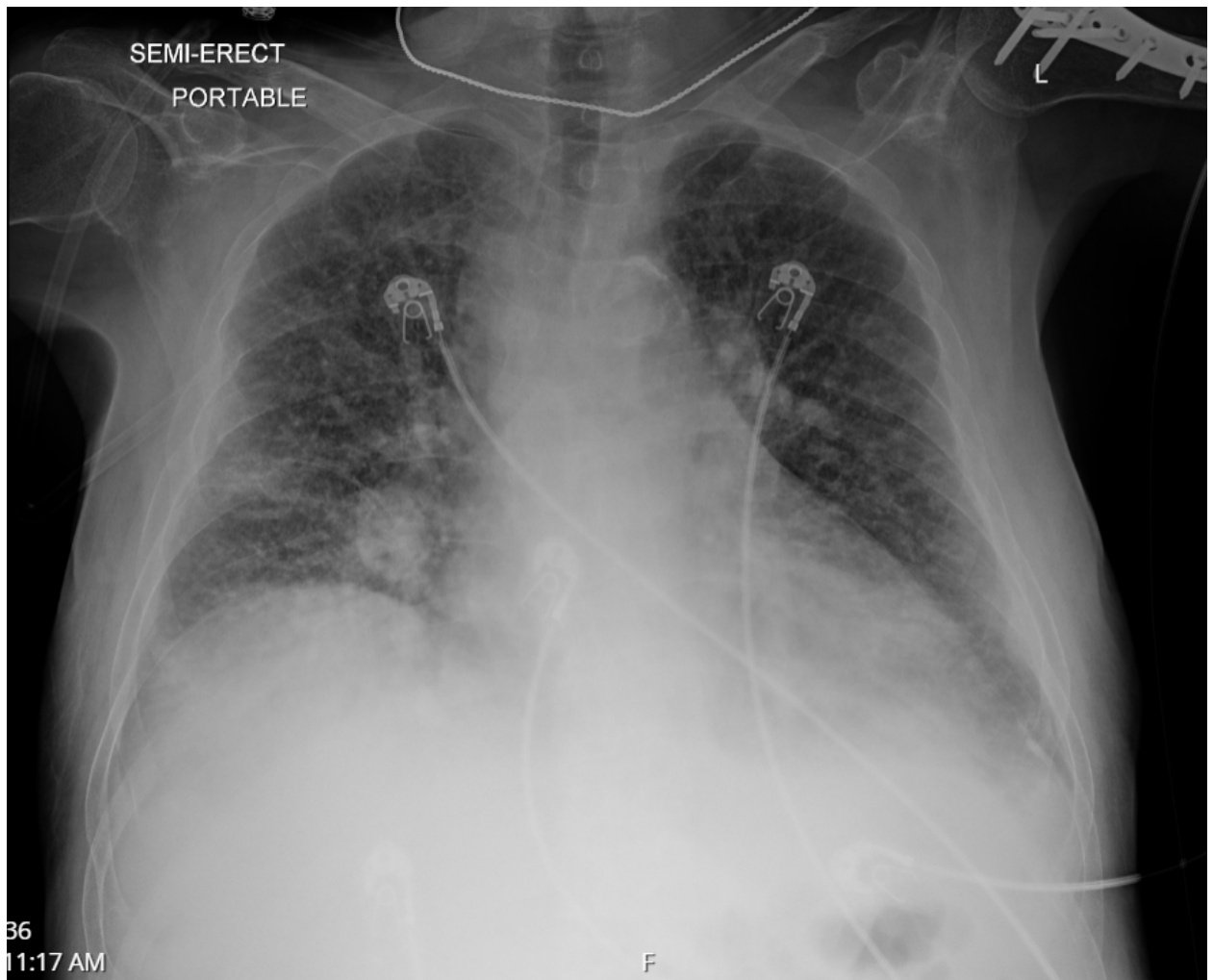
✓ Electrocardiogram:

- **Rate & Rhythm:** Regular, 100 b/m
- **P wave:** Present
- **QRS:** Narrow <0.12 SEC
- **Axis:** Normal
- **Evidence of ischemia or infarct** No ST elevation or ST depression.
- Tall R wave in V5-V6 concerning for LVH.



➤ CXR 02/14/2026:

- **Airway:** Trachea midline
- **Breathing:** Diffuse B/L interstitial/perihilar opacities consistent with pulmonary edema.
- **Cardiac:** Enlarged cardiac silhouette (cardiomegaly).
- **Diaphragm:** Mild costophrenic angle blunting.
- **Everything else:** Monitoring leads present. Post-traumatic deformity left clavicle. ORIF left proximal humerus/neck



➤ **TRANSTHORACIC ECHO (TTE) COMPLETE 01/09/2026**

- Left ventricle cavity size is normal. Left ventricular systolic function is mildly decreased with an ejection fraction of 40-45%.
- LV global hypokinesis is present.
- Left ventricle mild concentric hypertrophy.
- Right ventricle cavity is normal. Right ventricular systolic function is low normal.
- Mitral valve demonstrates moderate regurgitation.
- Tricuspid valve demonstrates moderate to severe regurgitation.

Left Ventricle

Left ventricle cavity size is normal. There is mild concentric hypertrophy. Systolic function is

mildly decreased with an ejection fraction of 40-45%. LV global longitudinal strain is reduced. Global LV hypokinesis is present. There is Grade I (mild) diastolic dysfunction.

Right Ventricle

Right ventricle cavity appears normal. Systolic function is low normal.

Left Atrium

Left atrium cavity is severely dilated. Left atrium volume index is moderately increased.

Right Atrium

Right atrium cavity is moderately dilated.

Mitral Valve

The leaflets are mildly thickened. The leaflets are calcified. There is moderate regurgitation. There is no evidence of mitral valve stenosis.

Tricuspid Valve

Tricuspid valve structure is normal. There is moderate to severe regurgitation. There is no evidence of tricuspid valve stenosis.

Aortic Valve

The aortic valve is trileaflet. There is mild regurgitation. There is no evidence of aortic valve stenosis.

Pulmonic Valve

There is no regurgitation or stenosis.

Ascending Aorta

The aorta appears normal in size.

Pericardium

Pericardium appears normal. There is no pericardial effusion.

➤ **CT CHEST WO IV CONTRAST 01/23/2026:**

- Acute appearing fracture of the posterior left 7th rib. Multiple chronic appearing bilateral rib fractures are noted.
- No pneumothorax or significant lung infiltrate.
Mild scarring/atelectasis of the lungs. Minimal left-sided pleural effusion. Small pericardial effusion.
- Severe coronary artery calcifications.
- Mild thoracic lymphadenopathy. Follow-up is recommended.

- indeterminate cystic lesions of the right kidney. Non-emergent ultrasound follow-up recommended.
- Lungs and pleural spaces: There is no pneumothorax or significant lung infiltrate. Mild scarring/atelectasis of the lungs noted. Minimal left-sided pleural effusion is seen. There is no right-sided pleural effusion. No mass.
- Heart: Small pericardial effusion is noted measuring approximately 12 mm greatest thickness. Severe coronary artery calcifications are noted. The heart is borderline enlarged.
- Bones/joints: There is acute appearing fracture of the posterior left 7th rib. Chronic appearing fractures of the lateral left 7th, 8th, 9th, and 10th ribs noted. Chronic appearing multilevel right lateral rib fractures noted. Chronic scoliotic curvature of the spine noted. The vertebral body heights and alignment are maintained. The sternum appears intact. The visualized clavicles appear intact. Chronic fixation hardware is associated with the visualized proximal left humerus. Degenerative changes of the shoulders are noted.
- Soft tissues: No acute abnormality of the chest wall.
- Vasculature: Calcific plaquing of the thoracic aorta is seen without appreciable aneurysm.
- Lymph nodes: Multiple mildly enlarged thoracic nodes are seen, largest is seen in the right paratracheal region measuring 17 mm short axis.
- Kidneys and ureters: Indeterminate cystic lesions of the right kidney, largest measures approximately 3.9 cm. Non-emergent ultrasound follow-up recommended.

➤ **TRANSTHORACIC ECHO (TTE) COMPLETE 07/22/2024**

- A complete transthoracic echocardiogram (including 2D, color flow Doppler, spectral Doppler, M-mode and strain imaging) was performed using the standard protocol. The study quality was good and diagnostic for the question posed.
- The left ventricle is normal in size. There is mildly increased left ventricular wall thickness consistent with mild concentric hypertrophy. LV IVS measures 1.2 cm. LV posterior wall measures 1.0 cm. There are no segmental wall motion abnormalities. Normal left ventricular ejection fraction. The left ventricular ejection fraction by visual estimate is 55% to 60%. The average global longitudinal peak systolic strain is borderline. The average global longitudinal peak systolic strain is 17.2% (absolute value).
- Grade I (mild) LV diastolic dysfunction. The estimated LV filling pressure is normal. There is reduced tissue Doppler velocity. E/A ratio is 0.96.
- The right ventricle is normal in size. There is borderline normal function of the right ventricle.
- The left atrium is normal in size.
- The right atrium is normal in size.
- There is trivial mitral valve leaflet thickening. There is normal mitral valve leaflet mobility. There is trace mitral regurgitation. There is no mitral stenosis.

- The aortic valve is trileaflet. There is mild aortic valve thickening. There is no aortic stenosis. There is no aortic regurgitation.
- The tricuspid valve is normal in structure. There is trace tricuspid regurgitation. Pulmonary artery systolic pressure measures 31 mmHg. The pulmonary artery systolic pressure is normal.
- The pulmonic valve is normal in structure. There is no mid-systolic notching of the RVOT VTI.
- The proximal segment of the ascending aorta is mildly enlarged.
- Trivial pericardial effusion present. Pericardial effusion consists of fluid. No chamber collapse present. There is no echocardiographic evidence of cardiac tamponade.
- The inferior vena cava is normal in size (diameter <21 mm) and decreases >50% in size with inspiration, suggesting a normal right atrial pressure of 3 mmHg (range 0-5 mm Hg).
- There is no prior study available for comparison at this organization.

Mild left ventricular hypertrophy. EF 55-60%. Mild diastolic dysfunction with normal left-sided filling pressure. Borderline LV strain. RV within normal limits. No significant valvular abnormalities. PASP 31 mmHg. Normal right atrial pressure, collapsing IVC. Trivial pericardial effusion without evidence of increased pericardial pressure.

Study Findings

Left Ventricle

The left ventricle is normal in size. There is mildly increased left ventricular wall thickness consistent with mild concentric hypertrophy. LV IVS measures 1.2 cm. LV posterior wall measures 1.0 cm. There are no segmental wall motion abnormalities. The left ventricular ejection fraction by visual estimate is 55% to 60%. LVEF by 3D echocardiography was assessed but found to be technically limited in nature. Normal left ventricular ejection fraction. The average global longitudinal peak systolic strain is borderline. The average global longitudinal peak systolic strain is 17.2% (absolute value). Grade I (mild) LV diastolic dysfunction. The estimated LV filling pressure is normal. There is reduced tissue Doppler velocity. E/A ratio is 0.96.

Right Ventricle

The right ventricle is normal in size. There is borderline normal function of the right ventricle. Right ventricular TAPSE measures 1.6 cm.

Left Atrium

The left atrium is normal in size. Left atrial volume index is 32 mL/m².

Right Atrium

The right atrium is normal in size.

Mitral Valve

There is trivial mitral valve leaflet thickening. There is normal mitral valve leaflet mobility. There is no mitral stenosis. There is trace mitral regurgitation.

Aortic Valve

The aortic valve is trileaflet. There is mild aortic valve thickening. There is no aortic stenosis. There is no aortic regurgitation.

Tricuspid Valve

The tricuspid valve is normal in structure. There is no tricuspid stenosis. There is trace tricuspid regurgitation. Pulmonary artery systolic pressure measures 31 mmHg. The pulmonary artery systolic pressure is normal.

Pulmonic Valve

The pulmonic valve is normal in structure. There is no mid-systolic notching of the RVOT VTI. There is no pulmonic stenosis. There is trace pulmonic valve regurgitation.

Aorta

The aortic root is normal in size. The proximal segment of the ascending aorta is mildly enlarged.

Pericardium

Trivial pericardial effusion present. Pericardial effusion consists of fluid. No chamber collapse present. There is no echocardiographic evidence of cardiac tamponade.

Interatrial Septum

No obvious interatrial communication noted with color Doppler imaging. There is excess mobility of the interatrial septum.

IVC/SVC

The inferior vena cava is normal in size (diameter <21 mm) and decreases >50% in size with inspiration, suggesting a normal right atrial pressure of 3 mmHg (range 0-5 mm Hg).

➤ NM HEART PHARMACOLOGIC STRESS SPECT MULTIPLE 12/12/24

- A perfusion 1-day rest/stress protocol was performed on a conventional SPECT camera.

- A pharmacologic stress test was performed using regadenoson.
- There were no significant ECG changes during vasodilator stress.
- Myocardial perfusion is abnormal.
- The stress end diastolic cavity size is normal.
- Global left ventricular function during stress is normal. The stress imaging left ventricular ejection fraction is 51%.
- Compared to the prior study of 6/13/2023, there are no significant changes.
- Under Pennsylvania state law, this patient will be notified by mail that further discussions of abnormal myocardial perfusion identified on this study with the ordering provider would be warranted and beneficial.

- Impression -

No evidence of ischemia. Moderate size, severe, fixed inferior defect consistent with infarction in the RCA distribution.

Normal left ventricular systolic function. Inferior wall is akinetic.

➤ **NM HEART PHARMACOLOGIC STRESS SPECT MULTIPLE of 06/13/23**

- Interpretation Summary -

- A perfusion 1-day rest/stress protocol was performed on a conventional SPECT camera.
- A pharmacologic stress test was performed.
- There were no significant ECG changes during vasodilator stress.
- Myocardial perfusion is abnormal.
- The stress end diastolic cavity size is normal.
- Global left ventricular function during stress is normal. The stress imaging left ventricular ejection fraction is 70%.
- Compared to the prior study of 5/7/2019, there are no significant changes.
- Under Pennsylvania state law, this patient will be notified by mail that further discussions of abnormal myocardial perfusion identified on this study with the ordering provider would be warranted and beneficial.
- There is a left ventricular defect that is medium in size with severe reduction in uptake present in the mid to basal inferior location(s) that is fixed.

There is a left ventricular defect that is medium in size with severe reduction in uptake present in the mid to basal inferior location(s) that is fixed.

Global left ventricular function during stress is normal. The stress imaging left ventricular ejection fraction is 70%.

Compared to the prior study of 5/7/2019, there are no significant changes.

Objective Data:

Vital Signs:

- **BP:** 159/65
- **Pulse:** 79
- **Temp:** 36.6
- **Resp:** 18
- **SpO2:** 98 on NC
- **Wt (today):** 135 lbs
- **Admission wt:** 150 lbs

PE:

- **Appearance:** He looks ill, not in acute distress
- **HENT:** Normocephalic and atraumatic. Eyes: Conjunctivae normal. Sclera anicteric. Neck: Mild elevated JVD.
- **Cardiovascular:** Regular rate and Rhythm, no murmur heard, Carotid pulses 2+ B/L without bruits. Capillary refill less than 2 sec. Trace pitting edema on legs B/L.
- **Pulmonary:** Breath sounds decreased at lower base B/L
- **Abdominal:** Soft- non-tender, not distended and normal bowel sounds.
- **Musculoskeletal:** Full ROM. Trace pitting edema on legs B/L
- **Skin:** Dry and Warm,
- **Neurological:** He is alert and oriented to person, place, and time.
- **Psychiatric:** He looks anxious.

ASSESSMENT:

1- Acute on Chronic Heart Failure with Mildly Reduced Ejection Fraction (LVEF 40-45%)-Ongoing

Data

- **TTE OSH (1/9/26):** LVEF mildly decreased with global hypokinesis 40-45%. Moderate mitral valve regurgitation. Moderate to severe tricuspid regurgitation.

- NT proBNP (2/14/26): 30, 940

Plan:

- Continuous telemetry
- Maintain K⁺ > 4, Mg⁺⁺ > 2
- Strict intake/output, daily weights, sodium/fluid restriction
- Rapid Effective Diuresis (RED) Pathway: completed
- Diuretic therapy: HOLD today due elevated creatinine.
- GDMT with:
 - Beta blocker: Bisoprolol 2.5 mg
 - ACEi/ARB/ARNI: Entresto 97-103 mg every 12 hours
 - Aldosterone antagonist: N/A
 - SGLT2i: empagliflozin on hold
 - Isosorbide/ hydralazine: Hydralazine 75 mg every 8 hours, isordil 5 mg twice daily
- Iron Deficiency: transferrin sat: 21%, ferritin >300
- Inotrope: N/A

2- Urinary retention- Improving

Data: Patent unitary retention and Bladder scan done, now s/p cudae catheter insertion by nursing on 02/17; was removed this morning (02/20), Void trial

Plan:

- Urology consulted, appreciate recommendations
- Continue tamsulosin 0.4 mg PO daily
- Bowel regimen

3- Hypocalcemia- ongoing

Data:

- Calcium on admission: 7.6, ionized calcium 0.91
- Today's calcium: 8.3

Plan:

- Alendronate not in formulary, continue oyster shells (calcium with vitamin D) 1 tab twice daily (was started on 2/15 after consulting with pharmacy)

4- CAD, HLD- Chronic, stable:

Plan

- Continue ASA 81 mg daily and atorvastatin 80 mg daily bedtime
- Continue ezetimibe 10 mg PO daily

3- HTN Chronic, stable:

Plan:

- GDMT as above
- Vital signs every 4 hours

3- 6- Type 2 DM Chronic, stable:

Data:

- Hemoglobin A1C (11/26/2025): 7.1%; (02/14/2026): 7.4%

Plan:

- Lispro 10 units with meals discontinued on 2/16 for hypoglycemia 42; also, pt reports he doesn't take this much at home
- Accuchecks before meals/bedtime
- Consistent carbohydrate diet
- Consult Endocrinology, appreciate recommendations
- Continue lantus 13 units daily at bedtime
- Continue aspart 5 units with breakfast and 8 units with lunch and dinner; and 3 units daily at bedtime

7- CKD stage IV Chronic, stable

Data:

- Baseline creatinine: ~ from recent review 2.13-2.30
- Creat on arrival: 2.18, Creat today: 2.6

Plan:

- Avoid nephrotoxins
- Monitor of BMP

8- COPD, Mild Thoracic Lymphadenopathy, Former Smoker- Ongoing

Data:

- Centrilobular Emphysema on 2L-4L @ Home Oxygen at Night time
- OSA not on CPAP
- **PFTs 2/11/26:** completed
- **CT chest 1/23/26:** Acute fracture posterior left 7th rib. Multiple chronic appearing bilateral rib fractures are noted. No pneumothorax or significant lung infiltrate. Mild scarring/atelectasis of the lungs. Minimal left-sided pleural effusion. Small pericardial effusion. Severe coronary artery calcifications. Mild thoracic lymphadenopathy. Follow-up is recommended. Indeterminate cystic lesions of the right kidney. Non-emergent ultrasound follow-up recommended.
- Mediastinal lymphadenopathy suspicious for sarcoidosis, other etiologies should be investigated including lymphoma from pulmonary note on 1/28/2026
- **CXR on admission (01/22/2026):** Rotated lordotic film. Probable mild interstitial edema. Right infrahilar nodular opacity most likely the pulmonary artery accentuated by positioning. Confirm on repeat chest x-ray. Mild cardiomegaly. Post-traumatic deformity left clavicle. ORIF left proximal humerus/neck
- S/P duo nebs in the ER, received 1 dose of prednisone 50 mg.

Plan:

- Continue albuterol and ipratropium nebs q 12 hours
- His home Breztri is not on formulary, pharmacy recommending Spiriva and Advair 250/50 1 puff every 12 hours; (with being on standing ipratropium, will not order spiriva at this time)
- Continue supplemental oxygen for POX goal of 88%-92%
- Nicotine patch while in the hospital
- Scheduled for PET Scan on Monday, will need to reschedule due to hospitalization

9- Recent posterior left 7th rib fracture, 1/23/2026- Improving

Data:

- Patient had rib fracture, osteoporosis, hx of falls, and recent admission to Nazareth Hospital 1/22/26-1/27/26 with rib fractures.
- Patient fell while walking backward.

Plan:

- Alendronate not in formulary, oyster shells (Calcium and vitamin D), as above
- Continue pain control with PRN tylenol and oxycodone
- add bowel regimen: senna tabs every 12 hours

Global

- **Code Status:** Full
- **Diet:** Consistent carbohydrate
- **DVT Prophylaxis:** Heparin 5,000 units Subcutaneous Q8h
- **PT/OT:** recommendations pending
- **Patient/family Interaction:** Patient's wife reports that his health has been declining
- **Discharge Disposition:** pending until completion of medical treatment.

Progress notes 2

HPI:

J.P is a 65 year old man with a history of HFpEF (50-55%), CAD, hypertension, HLD, type 2 diabetes, On 02/17 he presented with several weeks of chest pain, worsening over the last 3 days and dyspnea with CT evidence of pulmonary edema admitted to cardiology service for heart failure exacerbation.

History Interval:

Patient reports breathing feels improved compared to yesterday. He reports SOB at rest and on exertion on and off. He offers no complaints of chest pain, palpitation, or lightheadedness

- 24-hour weight change: down 1 lbs
- Weight today: 185 lbs
- Intake/output: 24-hour: - 1.6 L; since admission: - 3.1L
- Electrocardiogram today: n/a
- Blood pressure trend: 149/82
- Creatinine today: 1.5 (yesterday: 1.72)

Subjective data:

Allergies: NKA

Surgical history:

- PR CATH PLACEMENT & NJX CORONARY ART ANGIO IMG S&I 2024

ROS:

- **General:** Denies chills or fever or loss of appetite
- **HEENT:** Denies headache, No blurred vision or
- **Respiratory:** Reports SOB at rest and on exertion; at home he used two pillows while sleeping. No hemoptysis, sputum production, and wheezing.
- **Cardiovascular:** Reports chest pain on exertion that last for 10-15 mins prior to admission, no palpitations, or PND, no syncope.
- **Gastrointestinal:** No abdominal pain, diarrhea, melena, nausea, and vomiting.
- **Genitourinary:** Denies dysuria, urgency.
- **Musculoskeletal:** Reports leg swelling B/L and using cane at home. No hx of fall
- **Skin:** Denies itching or rash
- **Neurological:** Denies for dizziness and sensory change.
- **Psychiatric/Behavioral:** Denies anxiety and depression.

Home Medications:

- Amlodipine 10 mg PO Daily
- Aspirin (chewable) 81 mg PO Daily
- Atorvastatin 40 mg PO Daily
- Insulin Aspart (NovoLog vial) 8 units Subcutaneous TID with meals
- Insulin Aspart (NovoLog FlexPen) 8 units Subcutaneous TID before meals
- Losartan 50 mg PO Daily
- BD Pen Needle 31G x 5 mm – Use TID with meals
- Lancets – Use to check glucose 4 times daily
- Glucose test strips – Use to check glucose 4 times daily
- Glucometer kit – Use to check glucose 4 times daily

Hospital Medications:

- Acetaminophen 650 mg PO Q6H PRN
- Albuterol nebulizer 2.5 mg Neb Q12H
- Ascorbic acid 500 mg PO Daily
- Aspirin (chewable) 81 mg PO Daily
- Atorvastatin 80 mg PO QHS
- Bisoprolol 2.5 mg PO QHS
- Calcium–Vitamin D 1 tablet PO BID with meals
- Dextrose 12.5 g IV Q15 min PRN
- Dextrose oral gel 15 g PO Q15 min PRN
- Dextrose 25 g IV Q15 min PRN
- Dextrose oral gel 45 g PO Q15 min PRN
- Escitalopram 30 mg PO Daily
- Ezetimibe 10 mg PO Daily
- Ferrous sulfate 325 mg PO Every 2 days
- Fluticasone–Salmeterol inhaler 1 puff Inhalation Q12H
- Glycerin oral spray 2 sprays Q4H PRN
- Heparin 5,000 units SubQ Q8H
- Hydralazine 75 mg PO TID
- Insulin aspart (sliding scale) 0–12 units SubQ with meals & bedtime
- Insulin aspart 7 units SubQ with breakfast
- Insulin aspart 8 units SubQ with lunch
- Insulin aspart 8 units SubQ with dinner
- Insulin aspart 6 units SubQ QHS
- Insulin glargine 13 units SubQ QHS
- Ipratropium nebulizer 2.5 mL Neb Q12H
- Isosorbide dinitrate 5 mg PO BID
- Lansoprazole 30 mg PO Daily before breakfast
- Multivitamin 1 tablet PO Daily

- Nicotine patch 14 mg Transdermal Daily
- Oxycodone IR 5 mg PO Q4H PRN
- Polyethylene glycol 17 g PO BID PRN
- Sacubitril–Valsartan (Entresto) 97/103 mg PO Q12H
- Senna 8.6 mg PO Q12H
- Sodium bicarbonate 1,300 mg PO Q12H
- Sodium chloride nasal spray 2 sprays each nostril Q2H PRN
- Normal saline flush 0.5–50 mL IV PRN
- Normal saline KVO 0–10 mL/hr IV PRN
- Tamsulosin 0.4 mg PO QHS

Lab Results:

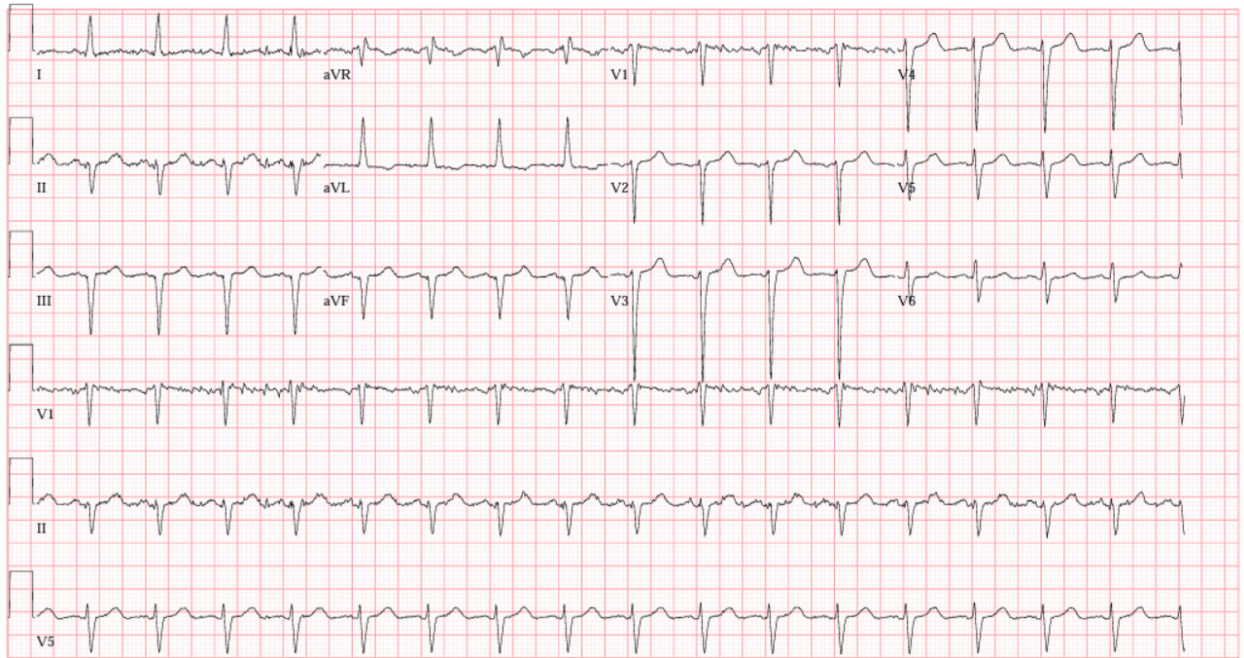
HS Troponin	9/20/2023: Troponin T, High Sensitivity (HS TNT) 21 2/17/2026: Troponin T, High Sensitivity (HS TNT) 125 2/18/2026: Troponin T, High Sensitivity (HS TNT) 135; Troponin T, High Sensitivity (HS TNT) 107																				
NT proBNP	2/17/2026: NT proBNP 14,832																				
Lipid Screen	Results for orders placed or performed during the hospital encounter of 02/17/26 LIPID SCREEN Collection Time: 02/18/26 7:02 AM <table><tr><td>Result</td><td>Value</td><td>Ref Range</td></tr><tr><td>Total Cholesterol</td><td>122</td><td>100 - 200 mg/dL</td></tr><tr><td>Triglycerides</td><td>80</td><td>25 - 150 mg/dL</td></tr><tr><td>HDL Cholesterol</td><td>46</td><td>40 - 59 mg/dL</td></tr><tr><td>LDL Cholesterol</td><td>60</td><td>0 - 129 mg/dL</td></tr><tr><td>Non-HDL Chol (Calc)</td><td>76</td><td>0 - 159 mg/dL</td></tr></table>			Result	Value	Ref Range	Total Cholesterol	122	100 - 200 mg/dL	Triglycerides	80	25 - 150 mg/dL	HDL Cholesterol	46	40 - 59 mg/dL	LDL Cholesterol	60	0 - 129 mg/dL	Non-HDL Chol (Calc)	76	0 - 159 mg/dL
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Hemoglobin A1C	2/19/2026: Hemoglobin A1c 6.6																				
TSH	2/19/2026: TSH 1.02																				
Iron Panel	Results for orders placed or performed during the hospital encounter of 02/17/26 IRON/ TRANSFERRIN/ TRNSAT Collection Time: 02/19/26 5:52 AM																				

	Result	Value	Ref Range
	IRON	52	45 - 182
			ug/dL
	TRANSFERRIN	240	180 - 329
			mg/dL
	TRANSFERRIN SATURATION	15 (L)	20 - 50 %
Ferritin	2/19/2026: FERRITIN 81.1		
LDH	2/18/2026: LACTATE DEHYDROGENASE Hemolyzed		

02/20/26 0600	02/19/26 1654	02/19/26 0552	02/18/26 1436	02/18/26 1127	02/18/26 0403	02/17/26 2316	
SODIUM	143	143	140	143	140	--	143
POTASSIUM	3.9	3.8	4.0	4.4	Hemolyzed*	--	3.7
CHLORIDE	105	101	102	104	103	--	104
CARBON DIOXIDE	29	32	28	28	32	--	26
UREA NITROGEN	27*	24*	23*	21*	20	--	24*
CREATININE	1.55*	1.72*	1.64*	1.42*	1.42*	--	1.51*
CALCIUM	8.9	8.6*	8.9	9.5	8.9	--	9.3
ALBUMIN	3.7	--	--	--	--	--	4.5
BILIRUBIN TOTAL	0.7	--	--	--	--	--	0.8
ALKALINE PHOSPHATASE	113	--	--	--	--	--	149*
ALT	42	--	--	--	--	--	80*
AST	24	--	--	--	--	--	40
GLUCOSE	117*	131*	209*	155*	213*	--	164*
BILIRUBIN DIRECT	0.2	--	--	--	--	--	0.2
WBC	5.6	--	6.2	--	--	--	--
HEMOGLOBIN	12.6*	--	12.8*	--	--	--	--
HEMATOCRIT	38*	--	40	--	--	--	--
PLATELETS	232	--	253	--	--	--	--
PTT	--	--	--	--	--	35.1	--
PT	--	--	--	--	--	14.7*	--
INR	--	--	--	--	--	1.3*	--
PRO BNP N TERMINAL	--	--	--	--	--	--	

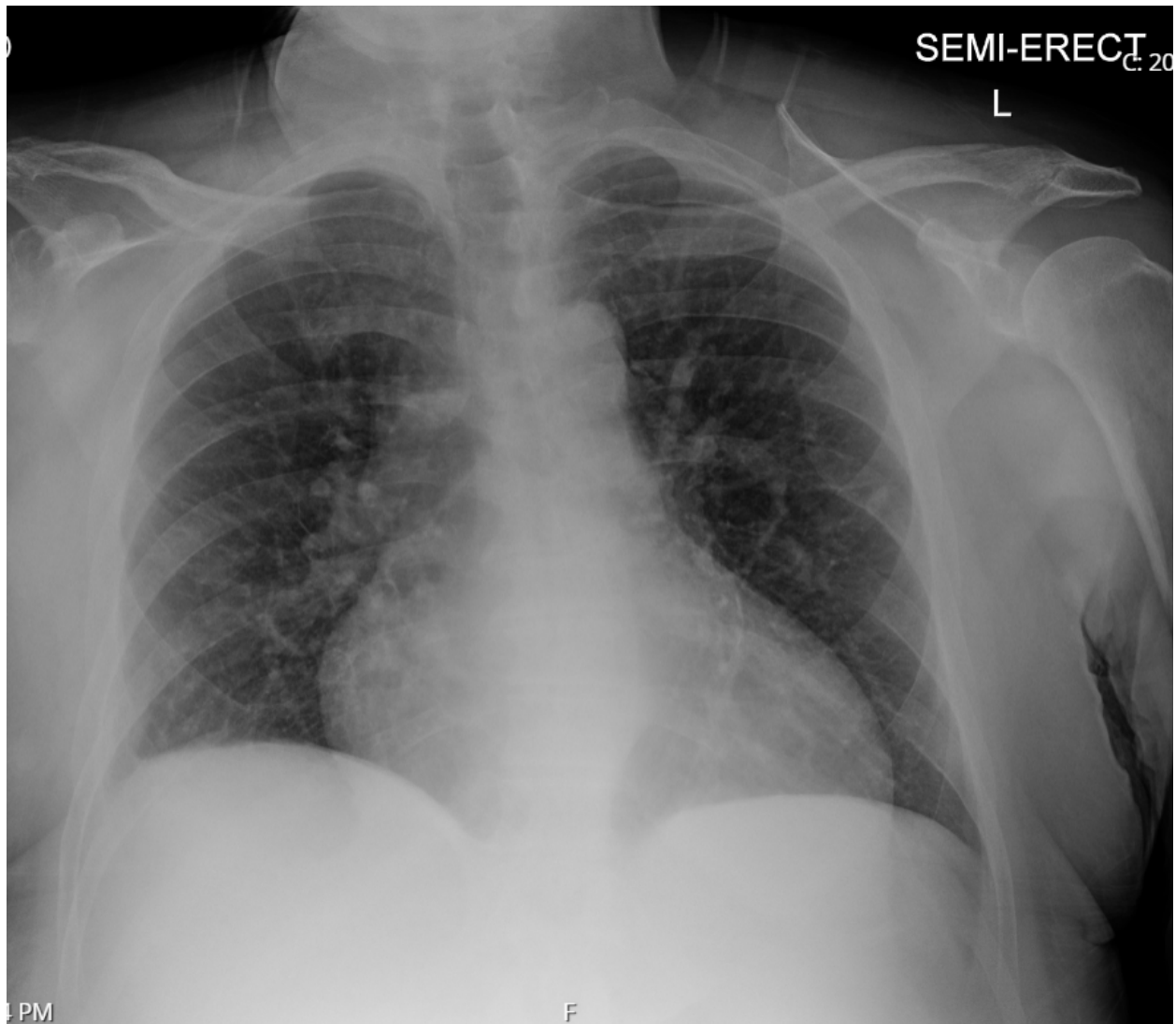
Related imaging:

- ✓ **ECG**
- **Rate & Rhythm:** Regular, 100-110
- **QRS:** Narrow < 0.12 sec
- **Axis:** left
- **Evidence of ischemia or infarct:** No ST elevation, No ST depression



➤ CXR 02/18/2026:

- **Airway:** Trachea midline. No deviation.
- **Breathing (Lungs & Pleura):** Lungs clear, no focal consolidation, no pneumothorax, costophrenic angles sharp; no obvious pleural effusion.
- **Cardiac:** Cardiac silhouette mildly enlarged indicates cardiomegaly
- **Diaphragm:** Both hemidiaphragms visible. No free air.
- **Everything Else:** No acute rib or clavicle fracture seen.



➤ **LHC 2024**

Coronary Angiography

1. Left main coronary artery: The LMCA gives rise to the left circumflex artery and left anterior descending artery. No LM disease.
2. Left circumflex coronary artery: The LCx gives rise to 2 obtuse marginal branches and terminates as a diminutive vessel in the atrioventricular groove. There is 50% mid Lcx disease.
3. Left anterior descending coronary artery: The LAD gives rise to large branching diagonal branch and multiple septal branches then bifurcates around the left ventricular apex. There is no coronary artery disease.
4. Right coronary artery: The RCA gives rise to a conus branch, 2 acute marginal branches, and a posterior descending artery before giving off multiple posterolateral branches. There is no coronary artery disease.

➤ Cardiac MRI with stress 2/29/24

1. Fairly extensive areas of subendocardial hypoperfusion on the stress perfusion images when compared with rest concerning for ischemia in the left anterior descending and circumflex territories. The delayed postcontrast images are suboptimal due to motion.

There is no definite areas of subendocardial delayed enhancement to suggest prior infarct. There is, however, diffuse wall motion abnormality with an ejection fraction of 39%. There is more focal hypokinesis of the anterior wall at the mid cavity towards the base with some myocardial thinning which may reflect hibernating myocardium. Findings are concerning for ischemic cardiomyopathy. Further evaluation with diagnostic cardiac catheterization is recommended.

2. A few small patchy areas of mid myocardial enhancement are seen in the septal and inferior walls. It is uncertain if these are artifactual or may reflect an underlying coexisting infiltrative disorder. There is no significant myocardial thickening.

➤ **CT chest with IV contrast:**

1. No pulmonary embolism on this moderate-quality exam.
2. Mild pulmonary edema and trace bilateral pleural effusions with associated atelectasis.

FINDINGS:

PULMONARY ARTERIES:

Image Quality is: Moderate, noting the following limitations: Streak artifact from dense contrast in the SVC. Pulmonary motion artifact.

No pulmonary embolism to the proximal subsegmental level.

LUNGS, PLEURA:

Airways: The central airways are patent, with normal course and caliber.

Lungs: Evaluation limited by pulmonary motion artifact. Mild diffuse intralobular septal thickening which likely reflects mild pulmonary edema. Mild scattered subsegmental and bibasilar atelectasis. No suspicious pulmonary nodule.

Pleura: Trace bilateral pleural effusions with associated atelectasis. No pneumothorax.

CARDIOVASCULAR, MEDIASTINUM, THYROID:

Cardiovascular and Mediastinum: Top normal heart size. No pericardial effusion. Mild coronary arterial calcifications.

Vasculature: Aorta is normal in course and caliber. The main pulmonary artery is mildly dilated to 3.4 cm which may indicate sequelae of pulmonary hypertension.

Visualized thyroid: The visualized portions of the thyroid gland demonstrate normal size and configuration. Partial calcified subcentimeter right thyroid nodule.

Esophagus: The esophagus is nondistended, precluding optimal assessment.

LYMPH NODES: There is no evidence for pathologically enlarged axillary, mediastinal or hilar lymph nodes by CT size criteria.

SKELETON, CHEST WALL: Mild bilateral gynecomastia. No suspicious osseous lesion. Mild multilevel spondylotic degenerative changes of the visualized spine.

UPPER ABDOMEN: The visualized upper abdomen is within normal limits. There is mild contrast opacification of the hepatic veins which could reflect heart failure.

Objective:

Physical Exam:

- **General:** Patient looks ill and not in acute distress
- **HEENT:** normal conjunctiva and mucus membranes moist. Neck: elevated JVD.
- **Chest:** clear to auscultation, no wheezing.
- **Heart:** Regular rhythm, normal S1, and normal S2. **Vascular:** delayed carotid upstroke, no carotid bruit, dorsal pedis diminished, radial pulses are palpable and symmetric +2. Capillary refill less than 2 sec
- **Abdomen:** soft, and non-tender, normal bowel sounds, no hernia
- **Ext:** Full ROM, leg edema B/L at least +2 pitting edema
- **Skin:** Warm and dry, no discoloration
- **Neuro:** alert, oriented x 3
- **Psychiatric:** Normal mood and affect

V/S:

BP: 149/85

HR: 100

Temp: 36.6

RR: 18

Spo2: 99% On RA

Admission Wt: 200 lbs

Today Wt: 185 lbs

Assessment&plan:

1- Acute on Chronic Heart Failure with Reduced Ejection Fraction (LVEF 20-25%), ACC/AHA Stage C, NYHA Class II, undifferentiated cardiomyopathy-ongoing DATA:

- **TTE (2/18/2026):** LV normal in size, mild LVH, LVEF 20-25%, RV mildly decreased systolic function, biatrial dilation, mild-moderate MR, severe low flow low gradient AS (although measurements may be inaccurate), moderate TR, PASP 63 mm Hg, small circumferential pericardial effusion, IVC normal.

- NT proBNP (2/17): 14,382

PLAN:

- Continuous telemetry
- Maintain K⁺ > 4, Mg⁺⁺ > 2
- Strict intake/output, daily weights, sodium/fluid restriction
- s/p Rapid Effective Diuresis (RED) Pathway
- Diuretic therapy: **IV furosemide 80 mg twice daily and add diuril 500 mg IV twice today**
- GDMT with:
 - Beta blocker: None due to concern for low output heart failure
 - ACEi/ARB/ARNI: Hold for now due to AKI; start low dose of sacubitril/valsartan
 - Aldosterone antagonist: will be added before discharge
 - SGLT2i: none; check cost
 - Isosorbide/ hydralazine: Hydralazine 75 mg every 8 hours
- Re-assess after 3 months maximally tolerated GDMT
- Iron Deficiency: Transferrin saturation 28%; ferritin 81.1, will order IV ferric gluconate 125 mg daily (for 8 doses or until discharge)

2- NSTEMI, CAD, non-obstructive- ongoing

Data:

- Predominantly Lcx disease (50%) on angiography from 2024 with otherwise clean coronaries. Reports chest pain occurs on exertion and resolves at rest.
- Troponin mildly elevated on presentation (107)

Plan:

- S/p 324mg ASA in the ED, continue aspirin 81 mg daily
- Repeat EKG with recurrent chest pain
- Continue home Atorvastatin 40mg daily
- Consider combined heart cath, Planned on next Monday
- Follow up LPa

3- AKI- ongoing

Data

- Baseline creatinine: 1.0-1.1
- Creatinine today: 1.5

Plan

- Continue IV diuresis
- Avoid nephrotoxins
- Daily BMP

4- Hypertension- Chronic, ongoing

Data:

- Home regimen: Amlodipine 10mg daily, Losartan 50mg daily

Plan:

- Hold Losartan for now given AKI, resume as renal function improves
- Hydralazine 50 mg every 8 hours
- Monitor BP

5- Type 2 DM Chronic, Stable

Data: Home regimen: Aspart 8 units three times a day with meals

Plan:

- Hemoglobin A1C (02/2026): 6.6%
- Medium-dose SSI with meals/bedtime
- Accuchecks before meals/bedtime

6- History of CVA- Chronic, Stable

- Continue aspirin 81 mg
- Continue atorvastatin 40 mg daily

7- Adrenal nodule, Thyroid nodule- ongoing

Data:

CT 02/18/2026:

- 13 mm low-attenuation right adrenal nodule, indeterminate etiology, probably representing adenoma.
- Prominent mediastinal and bilateral hilar lymph nodes, for example 7 x 14 mm right paratracheal lymph node on 2/75.
-

Plan:

- Adrenal nodules work up ordered
- OP Thyroid US
- **Global**
- **Code Status:** Full
- **Diet:** Consistent carbohydrate
- **DVT Prophylaxis:** Heparin 5,000-unit q8 hrs Subcutaneous
- **PT/OT:** N/A
- **Patient/Family interaction:** Explained current clinical status, treatment plan, and prognosis to the patient and his wife and daughter. All questions were addressed
- **Discharge Disposition:** Pending clinical improvement